Benefit Application

This medical policy relates only to the services or supplies described herein. Please refer to the Member's Benefit Booklet for availability of benefits.

Policy Statement

GEHA will provide coverage for surgical treatment for intraosseous radiofrequency ablation of the basivertebral nerve when it is determined to be medically necessary because the medical criteria and guidelines as documented below have been demonstrated.

When treatment for thermal destruction of the intraosseous basivertebral nerve is covered:

Thermal destruction of the intraosseous basivertebral nerve will be considered medically reasonable and necessary for the treatment of chronic lumbar back pain in patients who meet **ALL** the following criteria:

1. Chronic lumbar back pain of ≥6 months duration that causes functional deficit measured on a pain or disability scale*, AND
2. Documented failure to respond to ≥6 months of non-surgical management**, AND
3. Absence of non-vertebrogenic pathology per clinical assessment or radiology studies that could explain the source of the patient's pain, including but not limited to fracture, tumor, infection, or significant deformity, AND
4. Evidence of Type 1 or Type 2 Modic changes on MRI, such as inflammation, edema, vertebral endplate changes, disruption and fissuring of the endplate, vascularized fibrous tissues within the adjacent marrow, hypotensive signals (Type 1 Modic change), and changes to the vertebral body marrow including replacement of normal bone marrow by fat, and hypertensive signals (Type 2 Modic change), in 1 or more vertebrae from L3-S1.

NOTE: Thermal destruction of the intraosseous BVN must only be performed once per vertebral body from L3-S1 per lifetime. Up to 4 vertebral bodies may be treated during 1 procedure.

* Pain assessment and a disability scale must be obtained at baseline to be used for functional assessment.

** Non-surgical management may include but is not limited to:

- Avoidance of activities that aggravate pain;
- Trial of Chiropractic manipulation;
- Trial of Physical Therapy;
- Cognitive support and recovery reassurance;
- Injection therapy – epidural and/or facet;
- Spine biomechanics education;
- Specific lumbar exercise program;
- Home use of heat/cold modalities;